

Clinical Denial AR Audit Report

72-hour physician triage of a 47-case aged denial inventory. What is contestable, what is at deadline risk, and what to pursue first. No PHI required for this deliverable.

Specimen only. Fictional case. No PHI. No real patient, provider, or payer. Proprietary criteria content has been generalized. Clinical escalation methodology led by an MBBS physician with payer-side utilization-management review experience; clinical reasoning support only, not a licensed clinical opinion.

SECTION 01

Audit Scope

CASES REVIEWED	47 de-identified denial summaries
DATE RANGE	Denials aged 30–180+ days
TOTAL VALUE AT STAKE	\$1,274,000 (estimated from claim values provided)
PHI REQUIRED	No — audit conducted on de-identified denial rationale, service line, payer, and claim value
TURNAROUND	72 hours

SECTION 02

Triage Summary

PURSUE NOW 18 cases \$612,000	DEADLINE RISK 7 cases \$189,000
NEEDS RECORDS 12 cases \$318,000	DO NOT PURSUE 10 cases \$155,000

SECTION 03

Pursue-Now Cases (Top 5 by Value)

CASE	SERVICE LINE	VALUE	PHYSICIAN ASSESSMENT
#14	Inpatient sepsis, DRG downgrade 872 → 871	\$18,400	Clinical severity markers documented but not cited in prior appeal. Criteria-mapped argument constructable from existing record.

#03	SNF admission denial (MA)	\$22,600	Denial conflates acute stability with SNF-level need. Same analytical error seen in 4 other cases in this inventory. Strong appeal basis.
#27	Cardiac catheterization PA denial	\$41,200	Prior appeal used clinical narrative without criteria mapping. Indication documentation is strong. Needs criteria-mapped resubmission.
#31	Observation downgrade, chest pain	\$8,900	Two-Midnight Rule argument not used in prior appeal. Admitting physician documentation supports prospective expectation.
#09	IRF admission denial (MA)	\$34,800	Rehabilitation potential documented. Payer's denial cites maintenance-level plateau that is contradicted by therapy evaluations. Strong.

Deadline-Risk Cases

Seven cases have client-supplied appeal deadlines within the next 30 days (deadlines to be verified with the payer). Of these, five have contestable denial rationales but require immediate action:

CASE	DEADLINE	VALUE	ACTION REQUIRED
#08	12 days	\$31,400	Level 2 appeal deadline. Prior Level 1 used generic language. Needs physician escalation memo before submission.
#22	18 days	\$14,200	External review filing deadline. Case is contestable but requires documentation supplement (therapy notes missing from file).
#41	21 days	\$28,600	Level 1 appeal deadline. DRG downgrade case with strong clinical severity evidence. High priority.
#15	25 days	\$9,800	SNF continued-stay denial. Criteria-mapped argument needed. Moderate complexity.
#36	28 days	\$16,400	Observation downgrade. Two-Midnight Rule argument constructable. Needs physician attestation.

The remaining 2 deadline-risk cases (#19, #44) have denial rationales that are clinically correct. Filing an appeal would not change the outcome and would consume resources better spent on the 5 contestable cases above.

SECTION 05

Payer Patterns

Three patterns emerge across the 47-case inventory:

Pattern 1 — Post-acute stability conflation (11 cases, 3 payers): Multiple MA plans are applying the same analytical error across SNF and IRF denials — equating resolution of the acute condition with absence of post-acute skilled-care need. This is a systematic payer-side criteria misapplication, not a case-by-case clinical judgment. A template rebuttal addressing this specific logical error could be adapted across all 11 cases.

Pattern 2 — Observation downgrades on cardiac admissions (6 cases, 2 payers): Retrospective observation reclassification of chest pain and arrhythmia admissions. All six cases have admitting physician documentation supporting a Two-Midnight expectation. None of the prior appeals cited the Two-Midnight Rule framework. This is a systematic gap in your appeal team's argumentation, not a clinical documentation gap.

Pattern 3 — DRG downgrades without CDI challenge (5 cases, 1 payer): Five sepsis and heart-failure DRG downgrades where the paid DRG was accepted without clinical severity challenge. Combined revenue delta: approximately \$67,000. These are recoverable if the clinical severity evidence supports the higher-weighted DRG.

SECTION 06

Recommended Pursuit Sequence

Week 1: File the 5 deadline-risk contestable cases. Cases #08 and #41 are highest priority by deadline and value.

Week 2–3: Begin physician escalation on the 18 pursue-now cases, prioritized by value. Cases #27, #09, and #03 represent \$98,600 in combined client-supplied balances and have strong clinical basis.

Week 3–4: Request supplemental records for the 12 needs-records cases. Begin clinical review as records arrive.

Estimated recoverable: \$612,000 (pursue-now) + \$100,400 (deadline-risk contestable) + portion of \$318,000 (needs-records). These figures are client-supplied billed balances at issue in the flagged categories from a \$1.27M inventory — not a recovery estimate or a prediction of payment.

SECTION 07

Internal Pattern Tags

AR AUDIT

NO-PHI TRIAGE

47 CASES / \$1.27M

PAYER PATTERN: POST-ACUTE CONFLATION

PAYER PATTERN: OBSERVATION CARDIAC

PAYER PATTERN: DRG NO-CHALLENGE

Want this level of reasoning on your own cases?

START A 3-CASE EVALUATION

SEND A DE-IDENTIFIED CASE

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This specimen demonstrates the No-PHI Clinical Denial AR Audit — a 72-hour physician triage of aged denial inventory showing what is contestable, what is at deadline risk, and what to pursue first. No PHI required. Start by sending your de-identified denial inventory.

[Start a 3-case evaluation →](#) · [Send a de-identified case →](#)

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